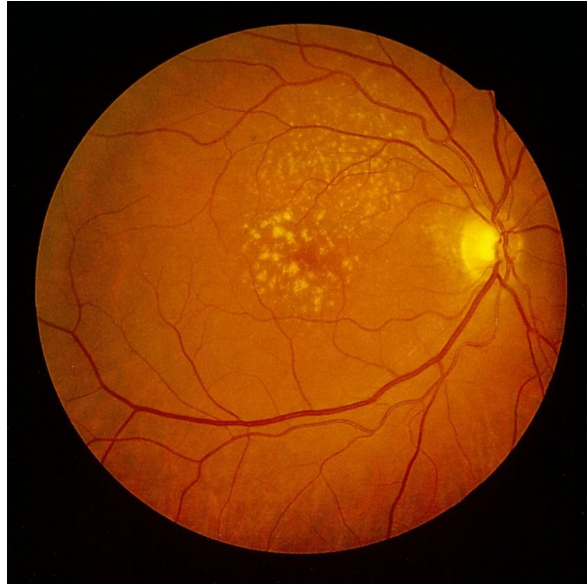


AMD: stage, activity and useful support

Clinical M3-M5 | Focused Type B notes

Prevention depends on stage

Drusen are a risk marker, not fluid



Yellow drusen cluster at the macula. Their presence does not by itself prove active neovascular AMD; assess stage and symptoms. Supplied M3 Anki, media 168.

Prevention choices

Situation	Decision
Early AMD	Risk-factor advice and symptom monitoring; AREDS2 is not universal.
Intermediate AMD or advanced AMD in one eye	Discuss appropriate AREDS2 supplementation.
Current/former smoker	Avoid beta-carotene-containing AREDS; choose AREDS2 if indicated.

Set expectations

Supplements reduce progression risk in selected patients. They do not replace symptom reporting, restore destroyed retinal tissue or guarantee prevention of neovascular disease.

References NEI: AREDS2 clinical trial findings | NICE NG 82: AMD management and monitoring

Activity determines whether treatment can help

Compare acuity, symptoms, fundus and OCT with the prior visit

Current finding	Clinical direction
New fluid or haemorrhage	Activity is suspected: reassess the anti-VEGF plan.
Stable inactive fibrosis, no prospect of functional improvement	Discuss treatment limits and rehabilitation.

Support continues when disease is inactive

Problem	Practical response
Reading/medication labels	Low-vision assessment, magnification and contrast aids.
Daily tasks	Rehabilitation and adapted routines.
New distortion after stability	Prompt re-evaluation; do not assume the old scar explains every new symptom.

Monitor both eyes

Review both eyes during AMD follow-up. A previously dry OCT is a dated observation; it cannot guarantee that the disease remains inactive at the next visit.